

Competitive environment and our advantage. The competitive environment mirrors the fragmented care-establishment pathway described above. Existing human and technology-enabled alternatives address individual steps or substantial portions of it, but generally operate through separate services, handoffs, and business models (Table 2). That fragmentation matters because responsibility can return to families between services, creating opportunities for care to fall through before it is established and feed the same vicious cycle.

Competitive category	Representative examples	Assess	Find	Fund	Execute	Establish	New workforce	Open access
Public information and benefits	Eldercare Locator; BenefitsCheckUp	•	•	•	•	○	○	•
General-purpose AI	ChatGPT; Claude; Gemini	•	•	•	•	○	○	•
Human navigation	Discharge planners; case managers; private care managers	•	•	•	•	•/•	○	•
Digital and hybrid navigation	Wellthy; Homethrive; Cariloop	•	•	•	•	•/•	○	•
Senior-care referral marketplaces	A Place for Mom; Caring.com	•	•	○	•	•	○	•
Workforce recruitment	Indeed; myCNAjobs; staffing agencies; referrals	○	○	○	○	○	•	○
Olera after the CRP	CareNavigator with Caregiver Staffing	•	•	•	•	•	•	•

Table 2. Competitive alternatives across the care-establishment pathway. • core capability · • partial or variable ○ not typical. New workforce means adding people to direct care rather than moving them between employers; open access means family access does not require institutional sponsorship and provider participation does not require payment for referrals.

Olera is designed from first principles backward from the endpoint that matters: established care before unmet need feeds the vicious cycle. The first step is to instrument the full pathway so CareNavigator can measure where, why, and how quickly care establishment succeeds or fails, and target interventions to the failure points and communities where they are needed most. Four architectural advantages follow.

Digital scale. The pathway can be screened and navigated across populations through the open web and mobile applications, with further distribution through institutional partners.

Open pathway. Families can enter without payment or institutional sponsorship and providers can participate without paying for referrals, preserving broad, neutral participation while giving the instrumented pathway visibility across more families and providers.

Demand and capacity. CareNavigator can connect providers at no referral cost with families whose needs, funding options, and service fit have been characterized. Caregiver Staffing can mobilize new, deliberately targeted labor pools rather than only redistribute scarce workers, while remaining independently useful to providers regardless of where their demand originates. Pathway data can further identify where workforce shortages are blocking care.

Accountability and evidence. Olera follows the case to the meaningful endpoint of established care rather than ending wherever an individual service's function ends, creating evidence about whether the pathway worked, where it failed, and how long it took.

Together, these advantages are designed to increase the effective capacity of the eldercare system to turn recognized needs into established care. For families, that means more pathways to funded services and established care; for providers, qualified demand and new workforce capacity; and for risk-bearing institutions, an evidence-generating intervention against the costly consequences of failed care establishment. The CRP tests whether these proposed advantages are real, measurable, and commercially meaningful.